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SFY 2026 Rate Development Components

Mercer Government

North Carolina Department of Health and Human Services
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A business of Marsh McLennan

1. Program Changes
2. Managed Care Adjustments
3. Medicaid Expansion Considerations
4. Unwinding Adjustment
5. Risk Adjustment
6. Next Steps

Agenda

Program Changes

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Program Changes

Comparison to SFY 2025 Programmatic Changes — Effective Prior to SFY 2024 Base Period

SFY 2025 Program Change	Effective Date	Adjusted for in SFY 2026 Rate Setting?
Reallocation of Duplicate Removal Adjustment	Resolved November 2023	No
AMH and CMHRP/CMARC Adjustment	July 1, 2021	No; full reliance on base data
Nursing Facility Reimbursement Changes	January 1, 2023	No; reflected in the base data
Home Health Electronic Visit Verification	February 1, 2023	No; reflected in the base data
Private Duty Nursing Fee Schedule Increases	July 1, 2023	No; reflected in the base data
Maternity-Related Service Reimbursement Changes	July 1, 2023	No; reflected in the base data

Program Changes

Comparison to SFY 2025 Programmatic Changes — Effective During or After SFY 2024 Base Period

SFY 2025 Program Change	Effective Date	Adjusted for in SFY 2026 Rate Setting?
Unbilled State Laboratory Claims	Ongoing	Yes
Opioid Treatment Program Changes	October 1, 2023	Yes; partial year
UNC Child and Adolescent Psychiatric Facility	November 1, 2023	Will be addressed in trend
Dental Procedures at an Ambulatory Surgical Center	January 1, 2024	Yes; partial year
BH Reimbursement Rate Increases	January 1, 2024	Yes; partial year
Physician-Administered Drug Program Fee Schedule Changes	February 1, 2024	Will be addressed in Rx trend
GLP-1 Coverage for Cardiovascular Risk Reduction	March 8, 2024	Yes; Mercer is also evaluating the impact of the sleep apnea indication approval for Zepbound®
Copay Waiver for NRT and Opioid Treatment	April 1, 2024	Yes
FQHC/RHC Reimbursement Changes	July 1, 2024	Yes
GLP-1 Coverage for Weight Loss	August 1, 2024	Yes
Mental Health Parity and Addiction Equity Act Compliance	January 1, 2025	Yes
ASAM SUD Service Array Changes	Various	Yes
Emerging Gene Therapy Considerations	Various	Yes

Managed Care Adjustments

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Managed Care Adjustments

Overview

- Adjustments for expected savings on medical costs relative to the base data period will be made.
  - As the SFY 2024 base data time period is assumed to fully reflect ultimate managed care savings levels relating to program launch (i.e., 100% phased-in according to the schedule), more targeted savings will be reviewed based on results of additional efficiency analyses.
  - These adjustments will include appropriate and attainable savings assumptions, as well as some portion of offsetting replacement costs for assumed preventable visits, as appropriate.
- Similar to prior years, Mercer will provide supplemental summaries to DHHS and PHPs related to the findings of these efficiency analyses. Additionally, further detail will be documented in the final SFY 2026 Rate Book.

Managed Care Adjustments

HOP Savings

- A retrospective analysis of medical costs for HOP users will be compared to a control group of individuals who were screened as eligible for HOP, but did not receive services during the data period.
- A comparison will be made to understand how medical costs changed for individuals once they began receiving HOP services compared to the control group.
- This review and potential adjustment will be applied, as appropriate, as to not duplicate efficiency expectations assumed in other analyses (e.g., LANE, PPA, Rx).

Managed Care Adjustments

LANE ED Analysis

- It is expected that the PHPs maintain clinical initiatives that aim to reduce unnecessary ED utilization, such as a visit for a condition that may not require emergency medical attention.
- Clinician-derived LANE diagnoses are identified in the encounter data alongside a target for the percentage of visits that may have been avoidable, depending on the diagnosis on the encounter.
- Only ED visits with a procedure code of 99281, 99282, or 99283 (i.e., excluding 99284 and 99285) are included in this analysis, given the likely lower complexity of the condition.
- An offset will be considered for appropriate replacement costs which are assumed to occur in lieu of the LANE ED visit.

Population	Total ER		Total Less Intensive LANE (99281-3)		Less Intensive LANE as a Percentage of Total ER	
	Visits	Dollars	Visits	Dollars	Visits	Dollars
	A	B	C	D	E = C / A	F = D / B
Aged, Blind, Disabled	185,238	$ 102,763,946	6,120	$ 2,035,332	3.3%	2.0%
TANF and Other Related Children (<1 Year)	125,863	$ 35,662,039	12,137	$ 3,920,603	9.6%	11.0%
TANF and Other Related Children (1 Years–20 Years)	886,586	$ 289,301,350	70,648	$ 22,562,557	8.0%	7.8%
TANF and Other Related Adults (21+ Years)	473,119	$ 222,977,401	19,929	$ 6,731,493	4.2%	3.0%
Non-Expansion Total	**1,670,806**	**$ 650,704,736**	**108,834**	**$ 35,249,986**	**6.5%**	**5.4%**

Managed Care Adjustments

PPAs

- This analysis is intended to identify and quantify the potentially unnecessary expenditures relating to inpatient hospital admissions.
- PPAs are identified based on the AHRQ PQIs and PDIs.
- Additional logic is applied to better identify the truly preventable admissions, such as enrollment duration thresholds and identification of more complex clinical conditions.
- Further analysis regarding inpatient readmissions is ongoing.

Population	SFY 2024 Hospital Inpatient		Final PPA After Exclusions		Percentage of Total Hospital Inpatient After Exclusions	
	Visits	Dollars	Visits	Dollars	Visits	Dollars
	A	B	C	D	G = C / A	H = D / B
Aged, Blind, Disabled	24,772	$ 517,763,096	3,563	$ 54,340,591	14.4%	10.5%
TANF and Other Related Children (<1 Year)	58,438	$ 467,035,719	96	$ 998,842	0.2%	0.2%
TANF and Other Related Children (1 Year–20 Years)	15,087	$ 195,446,228	1,215	$ 9,642,547	8.1%	4.9%
TANF and Other Related Adults (21+)	15,458	$ 232,465,021	1,501	$ 18,695,411	9.7%	8.0%
Non-Expansion Total	**113,755**	**$1,412,710,065**	**6,375**	**$ 83,677,391**	**5.6%**	**5.9%**

Managed Care Adjustments

Pharmacy Related Efficiency Analyses

- A retrospective analysis of pharmacy claims data will be performed to identify inappropriate prescribing and/or dispensing patterns.
- This includes a review of quantity limits, dosage limits, age edits, and therapeutic duplication as currently contemplated in the State Medicaid Direct system and therefore allowable to be managed by the PHPs through claims adjudication and prior authorization approval process.
- Generally, Mercer’s expectation is that these edits will be actively applied and managed by the PHP PBMs to align with the overall incentives in place for the PHPs to appropriately manage the care of their enrollees.
- State PDL requirements will be considered as part of these analyses.

Medicaid Expansion Considerations

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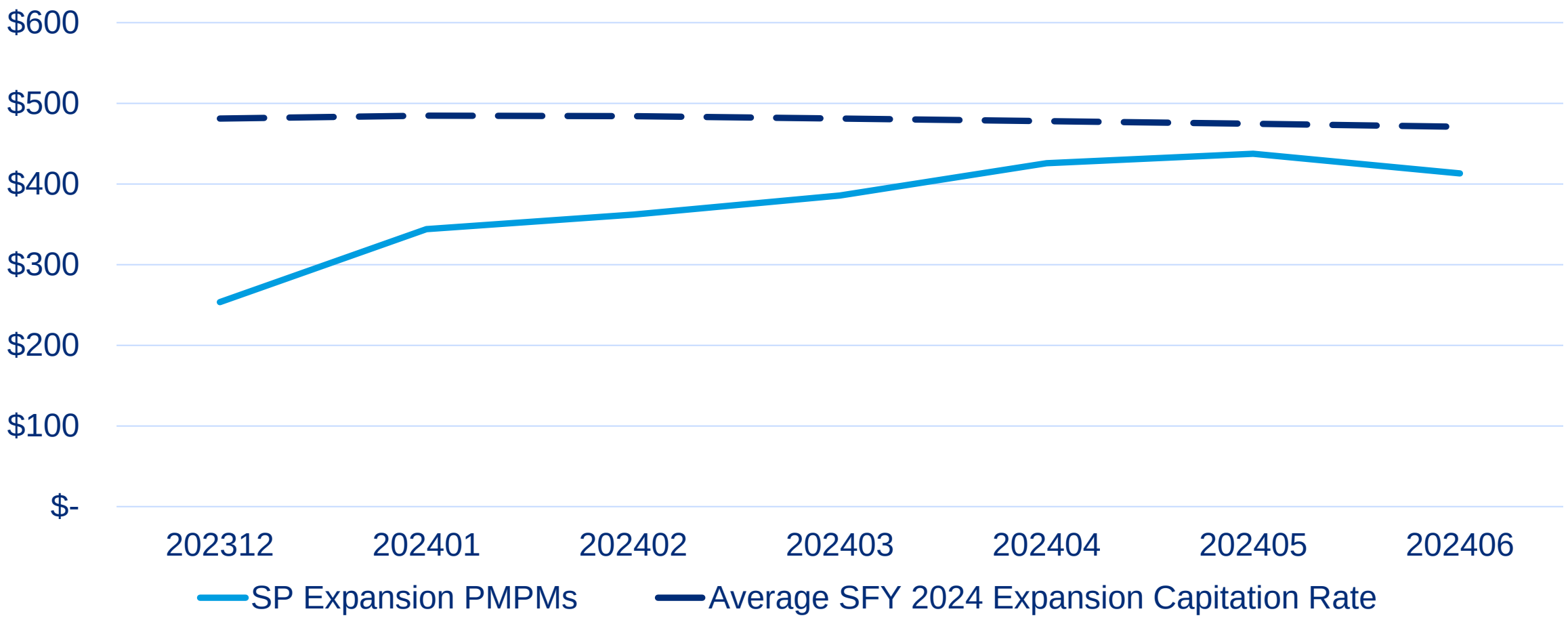
Medicaid Expansion Considerations

Overview

- General approach will be consistent with SFY 2025 Expansion rate-setting:
  - Proxy a baseline cost using SFY 2024 TANF adult data.
    - Acuity and demographic factors have been re-evaluated as described on subsequent slides.
    - Program changes and trend will continue to leverage assumptions for TANF adults.
    - Managed care assumptions will also be re-evaluated.
  - No adjustments will be applied for the impact of pent-up demand and TPL on the SFY 2026 rating period.
    - Upon review of emerging Expansion experience, Mercer has determined that the historical TANF Adult experience is representative of expectations for the future of the Expansion populations.
  - Consider emerging Medicaid Expansion enrollment.
  - Non-benefit load modeling will continue to be consistent with other Medicaid rate cells.

Standard Plan Expansion PMPMs

SFY 2024 Experience Comparison to Capitation Service Rate



Standard Plan Expansion

Demographic Acuity Adjustment Buildup

Row Label	Description	Newly Eligible, Ages 19 Years–24 Years	Newly Eligible, Ages 25 Years–34 Years	Newly Eligible, Ages 35 Years–44 Years	Newly Eligible, Ages 45 Years–64 Years
A	SFY 2024 TANF Female Base PMPM	$208.53	$288.07	$442.59	$737.07
B	SFY 2024 TANF Male Base PMPM	$139.12	$249.69	$399.45	$606.38
C	SFY 2024 TANF Female Percentage	59.8%	92.0%	84.2%	71.0%
D	December 2024 Expansion Female Percentage	53.8%	56.3%	59.7%	58.8%
E = Rows A and B weighted on Row D	Re-Weighted Expansion Overall PMPM	$176.46	$271.28	$425.20	$683.17
F	Additional Expansion Acuity Factor	4.6%	-3.2%	-2.2%	2.4%
G = E x (1 + F)	Final Expansion Overall PMPM	$184.52	$262.65	$415.98	$699.72
H = G / $344.67	SFY 2026 Demographic/Acuity Factor Compared to Overall TANF, Adult Base PMPM	0.54	0.76	1.21	2.03
I	*SFY 2025 Demographic/Acuity Rate Setting Assumptions*	*0.47*	*0.79*	*1.22*	*2.13*

Additional Expansion Acuity Factor (F) uses 50% weight on each of the SFY 2025 rate setting assumption and April 2024–June 2024 observed acuity. Care management and AMH base payments were excluded from this analysis.

Standard Plan Expansion

SFY 2024 Percentage of MMs with TPL Experience

Age	SFY 2024 TANF Adult Average TPL Penetration Rate	December 2023–June 2024 Expansion Average TPL Penetration Rate	June 2024 Expansion Average TPL Penetration Rate
19 Years–24 Years	1.6%	1.5%	1.3%
25 Years–34 Years	2.9%	2.2%	2.1%
35 Years–44 Years	3.6%	3.7%	3.5%
45+ Years	4.0%	4.6%	4.2%
Total Expansion	**2.7%**	**3.3%**	**3.0%**

- The table above shows that emerging SFY 2024 experience for the Expansion populations is well aligned with historical TPL levels for the TANF Adult population, which Mercer has observed to be stable based on comparisons to SFY 2022 experience.
- Therefore, Mercer will not be making any TPL adjustment for Expansion, as the TANF Adult base data is reflective of SFY 2026 expectations for Expansion TPL.

Unwinding Adjustment

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Unwinding Adjustment

Redetermination Data

- The Department performed research and analysis at the county level to understand the status of redeterminations and then used it to develop an enrollment projection at the county level which included estimated timing for completion of the backlog of redeterminations.
- Mercer reviewed these projections and determined them to align with expectations and understanding of the SFY 2024 enrollment period.
- Below is a summary of the unwinding status as of the mid-point of SFY 2024 and SFY 2026.
- Mercer is working to finalize regional acuity factors, which will leverage prior rate setting approaches with consideration for this emerging experience and enrollment alongside unwinding expectations provided by DHB.

	Region I	Region II	Region III	Region IV	Region V	Region VI	Statewide
Mid-SFY 2024	41.2%	24.6%	12.6%	14.8%	26.4%	31.6%	22.2%
Mid-SFY 2026	100.0%	96.9%	62.4%	86.9%	98.0%	100.0%	87.9%

Risk Adjustment

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Risk Adjustment

The TANF Child, TANF Adult, and Aged, Blind, and Disabled rate cells will continue to be risk adjusted, consistent with the SFY 2025 risk adjustment cadence and methodology.

DHB has made the decision to implement risk adjustment for the Newly Eligible Expansion populations prospectively, effective October 1, 2025.

Due to data timing restrictions, the SFY 2026a contract period will rely on a December 1, 2023–September 1, 2024 study period.

Scored criteria will be consistent with Non-Expansion risk adjustment, as individuals with 6 or more months of Medicaid eligibility within the 10-month study period.

Beginning SFY 2026b forward, the newly eligible study period will align with the non-Expansion rate cells.

Next Steps

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Next Steps

March 2025:

- Ongoing rate development
- All PHP feedback and considerations by March 13, 2025

April 2025:

- Rate meeting with Standard Plans (90 minutes)

May 2025:

- Rate finalization
- CMS rate certification package finalization

Appendices

Acronyms

- **AHRQ** — Agency for Healthcare Research and Quality
- **AMH** — Advanced Medical Home
- **ASAM** — American Society for Addiction Medicine
- **BH** — Behavioral Health
- **CMARC** — Care Management for At-Risk Children
- **CMHRP** — Care Management for High-Risk Pregnancies
- **CMS** — Centers for Medicare & Medicaid Services
- **COVID-19** — Coronavirus Disease 2019
- **DHB** — Division of Health Benefits
- **DHHS** — Department of Health and Human Services
- **ED** — Emergency Department
- **ER** — Emergency Room
- **FQHC** — Federally Qualified Health Center
- **GLP-1** — Glucagon-Like Peptide 1
- **HOP** Health Opportunities Pilots
- **I/DD** — Intellectual and Developmental Disabilities
- **LANE** — Low Acuity Non-Emergent
- **MLR** — Medical Loss Ratio
- **MM** — Member Month
- **NRT** — Nicotine Replacement Therapy
- **PBM** — Pharmacy Benefits Manager
- **PCP** — Primary Care Provider
- **PDI** — Pediatric Quality Indicators
- **PDL** — Preferred Drug List
- **PHP** — Prepaid Health Plan
- **PMPM** — Per Member Per Month
- **PQI** — Prevention Quality Indicators
- **PPA** — Potentially Preventable Admission
- **RHC** — Rural Health Center
- **SFY** — State Fiscal Year
- **SME** — Subject Matter Expert
- **SUD** — Substance Use Disorder
- **TANF** — Temporary Assistance for Needy Families
- **TPL** — Third-Party Liability
- **UNC** — University of North Carolina

Limitations and Caveats

Disclosures

This report is prepared on behalf of the State of North Carolina DHHS and is intended to be relied upon by DHHS for purposes of documentation of items under evaluation for Standard Plan capitation rate development for SFY 2025. It should be read in its entirety and has been prepared under the direction of Yemi Derebe, ASA, MAAA, Bob Karsten, ASA, MAAA, and David Salzwedel, ASA, MAAA, who are members of the American Academy of Actuaries and meet its US Qualification Standard for issuing the statements of actuarial opinion herein.

To the best of Mercer's knowledge, there are no conflicts of interest in performing this work.

The suppliers of data are solely responsible for its validity and completeness. Mercer has reviewed the data and information for internal consistency and reasonableness, but we did not audit it. All estimates are based upon the information and data available at a point in time and are subject to unforeseen and random events, and actual experience will vary from estimates.

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